

TRIDENT REGIONAL HOSPITAL

3600 Dock St, Norfolk, VA 23510 | (757)555-6610 | www.tridentrh.org

DISCHARGE SUMMARY

Patient Name: Kazimiera Kowalczyk	DOB: June 22, 1951 (Age 74)
MRN: MRN-5026891	Gender: Female
Admission Date: February 12, 2025	Discharge Date: February 25, 2025 (13 days)
Attending Physician: Dr. Hamid Karimi, MD — Hematology/Oncology	Insurance: No Insurance / Self-Pay
ICU Admission: YES — MICU/SICU	ICU Days: 3 of 13 total days

DIAGNOSES

Primary Diagnosis	ICD-10 Code	Status
Neutropenic Fever — Post-Chemotherapy Cycle 4	R50.81	Acute / Critical
Malnutrition — Severe Protein-Energy	E43	Chronic / Complicating
ESRD on Hemodialysis	N18.6	Chronic / Complicating

HOSPITAL COURSE

Kazimiera, a 74-year-old female with a history of Malnutrition — Severe Protein-Energy and ESRD on Hemodialysis, was admitted to the Neuro-ICU following witnessed generalised tonic-clonic seizure progressing to status epilepticus refractory to first and second-line benzodiazepines. The patient required propofol infusion for burst-suppression. EEG confirmed continuous electrographic seizures for > 30 minutes. Levetiracetam IV 3000 mg/day and lacosamide IV 400 mg/day were initiated. MRI brain identified a new cortical lesion requiring biopsy. The patient achieved seizure control by day 3 and was extubated on day 4. Neurosurgery and neuro-oncology consultations were obtained. Discharged on optimised anti-epileptic regimen with outpatient neuro-oncology follow-up.

VITALS AT DISCHARGE

■ **NOTE:** Several vital signs remain outside normal limits at time of discharge — close monitoring required.

BP	HR	SpO2	Temp	RR	Weight
115/57 mmHg	108 bpm	95% (on 3L O2)	100.4°F	24/min	98 kg

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Cefepime	2g IV	Every 8 hours	Completed in hospital — 7 days
Vancomycin	1.5g IV	Every 12 hours (AUC-guided)	14-day course
Metronidazole	500 mg	Every 8 hours	7-day course
Meropenem	1g IV	Every 8 hours	10-day course — completed in hospital
Vasopressin	0.03 U/min	Continuous IV infusion	Weaned and discontinued in hospital
Norepinephrine	Per protocol	Continuous IV infusion	Weaned and discontinued in hospital

FOLLOW-UP PLAN

URGENT: Hematology/Oncology: Dr. Hamid Karimi, MD — within 72 hours of discharge (February 28, 2025).

Primary Care: Within 1 week (March 04, 2025). Full labs including BMP, CBC, LFTs.
Home health nursing: Daily vitals and wound check for first 7 days post-discharge.
Inpatient rehabilitation / LTACH: Transfer arranged — starts March 05, 2025.

■ **HIGH-RISK DISCHARGE — Patient requires close monitoring. Re-admission risk: HIGH.**

Attending Physician Signature: _____

Dr. Hamid Karimi, MD — Hematology/Oncology | License #: VA-MD-10234 | Date: February 25, 2025

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